



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : مني محمد احمد ++
 المحافظة : القاهرة الحي / المدينة : طره
 رقم البطاقة : 28006180102926 رقم التليفون : 01220033729
 العمر (تاريخ الميلاد) : 18-06-1980 المهنة : ربه منزل
 العنوان : ++++ تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-
 الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
 نفقات العلاج في حالة ثبوت غير ذلك
 الإخصائي الاجتماعي (.....) التوقيع: (.....)
 المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي محوسب للصدر [10865] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
 الاسم : د/ندى حسن الوظيفة : التوقيع :
 الاسم : د/نرمين رمضان الوظيفة : التوقيع :
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
 بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

ختم
 شعار الجمهورية

مدير عام
 مديرية الشؤون الصحية

رئيس
 المجلس الطبي المتخصص

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Name: منى محمد احمد ++ National ID: 28006180102926	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي CT scan of the chest.
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بمركز لتأهيل المستعدين

منى
محمد احمد صليح

٢٣ شارع الوفاء والامل - طره الاسفنت
طره - القاهرة



JW2587971

٢٨٠٠٦١٨٠١٠٢٩٢٦



٢٠٢٣/٠٢ ٢٨٠٠٦١٨٠١٠٢٩٢٦

ربة منزل

متزوجة

مسلمة

انثى

شحاته محمود هيبه عبدانه

البطاقة سارية حتى ٢٠٢٣/٠٢/١٧





Patient's Name: mona mohamed ahmed
Date: 2026-03-30
Referred by: Prof.Dr. et cap iv
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

A known case of managed right breast cancer

Compared to last study dated 13-4-2025

I- CHEST:

- Right MRM showed clear operative bed with no enhanced focal lesions or fluid collection.
- The left breast showed UOQ soft tissue mass lesion shows progressive course a regard the size measures about 7 x 2.6 compared to 6.2x2.35 cm causing skin retraction and associated with enlarged ipsilateral axillary lymph nodes, the largest measures about 2x1.7 cm **for further sono-mammography**
- Bilateral mosaic parenchymal pattern with air trapping denoting small air way disease.
- Left apical and right lung lobes thick atelectatic bands are noted
- No pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.
- Enlarged right cupula of diaphragm.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous fatty parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.



Patient's Name: mona mohamed ahmed
Date: 2026-03-30
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Right caval lymph node measures about 16x11.7mm with multiple few aortic lymph nodes.
- No ascites.
- **The examined skeleton**
 - Progressive course as regard The previously noted sternum and right 1st rib soft tissue mass component measures about 6 x3 cm compared to 5x4.5 cm at sternum and 5.5x4.5 compared to 5.5 x4 cm at 1st rib.
 - Lytic lesions with sclerotic margins seen at cervical vertebral and D1,D2 vertebral bodies.
 - For further bone scan

Opinion:

- **STILL SEEN Bilateral mosaic parenchymal patterns with bilateral thick atelectatic bands.**
- **Few abdominal lymph nodes enlargement**
- **Progressive course as regard the Left breast mass lesions with enlarged axillary lymph nodes Sternum and right 1st rib lytic lesions with soft tissue component likely metastasis**

Dr.Norhan Ramzy,MD
Dr Abubakr Hejazi MSc FEVR

Dr.Nourhan Ramzy

Dr.Nourhan Ramzy

05/11/2026

MDT FORM

Name:	Monna Mohamed El-Sayed Othman	MRN:	4848
Age:	46 Y	ID:	28006180102926
Menopausal Sta	prem	Phone:	01104394198
NRS SCORE:	3	Marital Status:	
BMX:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:
ER: PR: HER2: KI67:

Sonomammography:

Mets Workup:

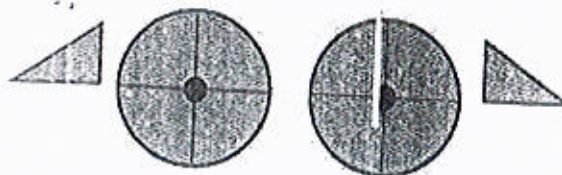
History:

premenopausal met breast cancer to lung lymph nodes recently newly developed bone mets ldc2 er+ve pr3/8 her2+ ki 35% .h story 2023 rt breast cancer ct4b n2 mo rec taxotere carboplatin x6 cycles no anti her2 given rt mrm 9/2023 ypt4 ypn3 postop cts newly dev rt internal mammary in left axilla. In pd of pulmonary nodules rec xeloda and herceptin from 10/2023 to 1/2024 then lost fu till 9/2024 pd so rt gemzar tykerb from 9/2024 to 2/2025 rec 7 cycles then lost fu till 8/2025 dp st taxol rec 22 weeks last 2/2025 cts on 30/3/2026 progressive course of left breast and axillary in pd of sternal and rt 1st rib soft tissue component and newly developed lytic lesions d1,d2 vertebral bodies for mdt bs sm requested

Recommendations:

MDT Outcome & Plan

Herceptin RECHALLENGE WITH XELODA
please check peripheral neuropathy ZOMETA EVERY 28 DAYS
CHECK FOR PALIATIVE RTH ON 1ST RIB FOR SOFT TISSUE COMPONENT



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ ابراهيم محمد في		د/ ماهر مختار			



Punjab Cancer Society
Ministry of Health and Family Welfare
Government of Punjab

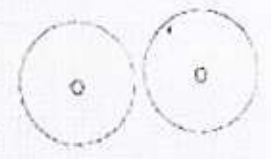
الجمعية السرطانية
وزارة الصحة والعائلة
الحكومة Punjab



A-Patient ID
1- Name: **میری محمد حسن عثمان**
2-10 number: **28006180102926**

MDT form
3- Medical number:
Female
Sex:
Patient Age: **43 Y**

83126563
Old Med:
596/2023



REASON OF MDT
Date of MDT
2025-05-13

Family history
marital status
Co morbidity
Phone
NRS Score
Menopausal status
BMI
GROUP
Initial STAGE

Clinical data

Breast Imaging :

BX IDC		ER 2/B	PR 3/8	Her 2 Neu 3	SISH	Ki67 35
T 4B		N 3		M 1		

Sonomammography

PATIENT HISTORY
PT RECEIVED 6 CYCLES TAXOTERE/CARBO ,,NO APROVAL FOR HP,,SUBJECTIVE
IMPROVMENT BUT
STILL BREAST LESION
RT MRM:
free margins
free nipple . areola
LN 13/15
partial pathological response
YPT4 YPN3
NEW CT CAP :PROGRESSION OF RT INT MAMM LNS, ENLARGED LEFT AXILLARY LNS,
RT SUPRACLAV LNS NEWLY DEVELOPED, LUNG NODULES
pt received 4c xeloda HERCEPTIN
Gemzar & Tykerb 7 CYCLES

MDT OUTCOME & PLAN

- For REBIOPSY
START AC UNTIL BX RESULTS

SIGNATURE
RADIOLOGY

Patient Name: Mona Mohamed Ahmed Othman	Date : 13/04/2025
Exam : C.T CHEST WITH CONTRAST	Age : 45 Year(s)
Medical ID: 596/2023	Pat. ID: 28132

**Post-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS
REVEALED: -**

*****As Compared to the previous study dated last study 08-12-2024,
current study shows:***

Chest:

- Right mastectomy showing no gross operative bed masses or collections.
- **Mixed course** regarding the size and increase in number of the enlarged left axillary lymph nodes, the largest measures about 5.1 9X3.5 cm compared to 4.5x3.6 cm and seen inseparable from the overlying skin.
- **progressive course** regarding the size of right internal mammary LNs, the largest measures 4X 3.4 cm compare to 1 cm currently infiltrating the sternum with extraosseous soft tissue component.
- **Progression in size of the Left upper lung lobe apico-posterior nodules** the largest measures 3.7X4.3 cm compared 2.7x2.2 cm previously.
- **Progression of enlarged right supraclavicular** the largest measures about 4.6x 2 cm with currently seen infraclavicular nodes ? nodular density
- **Still noted mildly enlarged right axillary lymph nodes.**
- **Newly developed** the right middle and lower subsegmental atelectatic bands
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.

Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions or biliary radicles dilatation
- Normal size & parenchymal texture of the spleen with no focal lesions seen, No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal non contrast appearance of the pancreas and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- **Stable** D6 vertebral body sclerotic area is seen with heterogenous osseous texture of the examined bones.

OPINION:

- Status post right mastectomy showing
 - Mixed course regarding left axillary lymph nodes

Patient Name: Mona Mohamed Ahmed Othman	Date : 13/04/2025
Exam : C.T CHEST WITH CONTRAST	Age : 45 Year(s)
Medical ID: 596/2023	Pat. ID: 28132

- Progression of pulmonary nodules and right internal mammary lymph nodes.
- Progression of right supraclavicular lymph nodes .

Dr. Fatima Ahmed

Prof. Menna Talla Hanafy, M.D.



Patient Name	: الأستاذ / منى محمد احمد عثمان	Request No.	: 13007874
Patient No.	: 28006180102926	Request Date	: 01/08/2024
Path No.	: 2696/24	Verification Date	: 07/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبداء
Age	: 44 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast malignant looking solid mass lesion.
Nature of Specimen: Tru-cut biopsy, collected at 1/8/2024.

Gross Examination:

- Multiple grayish white tissue cores up to 1.8 cm, totally submitted.

Microscopic Examination:

- * Histological type: Focally necrotized invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Negative.
- * Perineural invasion: Negative.
- * Angioinvasion: Negative.
- * Microcalcifications: Negative.

Diagnosis: (Revised)

Left breast malignant looking solid mass lesion, Tru-cut biopsy:
Focally necrotized invasive duct carcinoma, Grade II.
For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ghonaim.
Dr. Toka Medhat.

Verified by :
Dr.Toka Medhat

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / منى محمد احمد عثمان	Request No.	: 13007874
Patient No.	: 28006180102926	Request Date	: 01/08/2024
Path No.	: 2696/24	Verification Date	: 24/08/2024
Sex	: Female	Financial Category	: أورام دار السلام - مبالغه
Age	: 44 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast malignant looking solid mass lesion.
Nature of Specimen: Tru-cut biopsy, collected at 1/8/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in the tumor cells (score 0/8).
- PR: Weak nuclear reaction in 1% of tumor cells (score 2/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion:

Left breast malignant looking solid mass lesion, Tru-cut biopsy:
Focally necrotized invasive duct carcinoma, Grade II (revised).

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 2/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 25% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :

Dr.Toka Medhat

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Patient Name الأستاذة / منى محمد أحمد عثمان
Patient No. 28006180102926
Path No. 2696/24
Sex Female
Age 44 Y
Referring Doc.

Request No. 13007874
Request Date 01/08/2024
Verification Date 24/08/2024
Financial Category برامج دار السلام - ميائنة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghonaim.
Dr. Toka Medhat.

Verified by :
Dr. Toka Medhat

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